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Shabana Bharmal & Sanjiv Ahluwalia

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LEADING ARTICLE



Building belonging in medical education

Shabana Bharmal^a and Sanjiv Ahluwalia^b

^aDepartment of Community based medical education, Queen Mary University of London, London, UK; ^bSchool of Medicine, Anglia Ruskin University, Chelmsford, UK

ABSTRACT

Belonging is increasingly recognised as an important component of successful medical education. It affects academic achievement, wellbeing, and professional identity formation. This article examines how this concept applies to undergraduate students on community clinical placements and postgraduate vocational general practice (GP) trainees. Additionally, we consider the broader impact of belonging beyond the learner – on patient safety and quality of care. We explore how structural, cultural and relational factors can promote or hinder a sense of belonging. The shift towards competency-based training, reductions in continuity of care, and changes in the primary care infrastructure and workforce, complicate traditional notions of belonging. This highlights the need for holistic, flexible and inclusive educational practices. Key themes that promote belonging include the importance of connection through peer networks, mentorship, continuity, an inclusive learning environment which fosters psychological safety, and learner autonomy to accommodate differing needs and interests. Forming high quality relationships strengthens all of these. We propose centring a sense of belonging in medical education through a more relational approach. This can better support the development of resilient, reflective and connected general practitioners prepared to meet the evolving needs of primary care.

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Introduction

The traditional route to a career in general practice, focused on a vocational experience between GP trainee and GP trainer followed by entry into a partnership, has been steadily challenged over the past 15 years. Factors include moves towards competency-based training, pressures of healthcare commodification and business management in general practice, and diminishing resources at a time of rising demand. These have all contributed to changes in the primary care workforce, reductions in continuity of care, and an increasing sense of dissatisfaction with patient experience of general practice.

We now work in more diverse teams, with a much greater mixed economy of types of professionals who have a wider variety of working arrangements – this is the change that our patients, students, trainees and staff are experiencing. Reductions in continuity of care and its impact on patient outcomes make it necessary for us to reflect on the consequences of the changes wrought upon the clinical workforce in general practice. One response would be to argue for a rollback of the many changes implemented in the commissioning, delivery, and training of general practice over the last 20 years. This is neither feasible nor necessarily desirable. Another is to

consider the future of general practice (and its workforce) through the values that really matter. One of these (amongst many) is the sense of ‘belonging’ that patients and staff describe as being important. In this article, we therefore explore the meaning of belonging in general practice and its education, factors that affect this, and what this might mean for the future of medical education.

What is belonging in the context of general practice?

Belonging is a complex and multi-dimensional concept which lacks a single agreed-upon definition [1]. The psychologists Baumeister and Leary identified belonging as a fundamental need where human beings are deeply motivated to form and maintain ‘strong, stable interpersonal relationships’ [2]. Focusing on the learning environment, the educational literature builds on this concept with emphasis on the experience of feeling accepted, valued, included and supported by others [3,4]. This sense of connection is dynamic, occurring in overlapping academic, social, personal and physical spaces. It goes beyond feeling connected to peers, to also include staff and the wider learning environment and is

CONTACT Shabana Bharmal ✉ shabana.bharmal@qmul.ac.uk 📠 Queen Mary University of London

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shaped by contextual factors, past experiences and mental health [5]. In health professional education, Levett-Jones and Lathlean added that the degree to which an individual's professional and/or personal values are in harmony with those of the group is a key factor [6].

Belonging in medical education can be fostered across different levels: to a specific clinical rotation, to the wider academic community of the medical school or vocational training scheme (VTS), or at the broadest level, to the medical profession. In the context of the primary care clinical placement, an individual's sense of belonging is intrinsically linked to their experiential learning experiences of patient encounters, observing role models, practising their skills, receiving personalised feedback and reflecting on these activities. The organisational culture, interactions with people and the environment, in addition to individual level factors such as the learner's affective attributes all shape the extent to which it is felt.

Why is this important?

Belonging has a positive psychological impact [2]. It is linked with positive mental state, cognitive engagement and resilience. Conversely when there is a lack of belonging, through isolation or social exclusion, it results in negative psychological, behavioural, physical and mental health consequences [2].

These impacts shape the ability to engage with the learning process and influence academic outcomes. Learners with a greater sense of belonging are associated with higher levels of academic motivation [7], engagement [8], and improved academic success and retention [9].

Additionally, belonging plays an important role in professional identity formation, particularly for diverse identities and voices [10]. Mayer et al. highlight that for disabled healthcare learners, the social context and the legitimisation of an individual's professional role by the community of practice are crucial [11]. This is fostered through a supportive and inclusive environment which values the richness of diverse representation.

From an organisational perspective, staff working in a healthcare environment where they feel that they are valued, trusted, and can safely raise concerns, leads to higher-quality care and patient outcomes [12].

Factors influencing a sense of belonging in the NHS

The NHS faces numerous challenges that impact its ability to meet core needs to build belonging. There are ongoing workforce retention challenges with notable

stress and burnout in doctors [13]. Recent reports have highlighted the widespread issues of discrimination across the healthcare environment [14].

As such, it requires ongoing effort to foster and maintain belonging, ensuring that learners feel supported and engaged throughout their educational journey. Compassionate leadership can help promote an inclusive, person-centred culture that prioritises the fundamental needs of staff, which includes fostering belonging [15].

Psychological safety in the clinical learning environment – where individuals within a group feel able to take interpersonal risk and contribute without fear of negative consequences – increases learners' sense of belonging [16]. McClintock et al. identify that psychological safety is often lacking because of the systems and structures in the healthcare environment [16]. At the organisational level, the fast pace of work and high productivity pressures can erode into protected time for development or perceived support for work-life balance. Social hierarchy and power differentials within teams can adversely affect building relationships with teams and social cohesion. Observable behaviours that can promote psychological safety and, in turn, a sense of belonging include leadership being physically available, respectful interpersonal communications and providing learner autonomy.

Broader contextual matters may also influence the sense of belonging. The framing of the healthcare system as 'broken', a focus on efficiency and productivity over other aspects such as quality of care, and the burden of bureaucracy can all have the potential to form barriers to belonging.

Transactional and relational interactions in healthcare

According to Iles, interactions in healthcare can be broadly classified into transactional and relational ones [17]. Transactional interactions are characterised by formalised processes, standardised protocols, and a focus on measurable outcomes. Iles observed that healthcare has shifted from being a 'covenant of care' to a series of 'auditable transactions', where healthcare interactions are reduced to quantifiable exchanges. This model emphasises efficiency and accountability but under-values the individuality of patients and professional judgement of healthcare providers.

By contrast, relational interactions prioritise the quality of relationships among healthcare professionals and between caregivers and patients. Iles advocates for recognising the importance of these relationships in delivering compassionate and effective care. She

suggests that nurturing strong interpersonal connections within healthcare teams can lead to improved patient outcomes and a more supportive work environment.

It is our contention that both high quality transactional and relational interactions are required in healthcare. However, the focus on transactional interactions (thought to be easier to observe and measure) risks crowding out the benefits of relational interactions.

Importance of a relational approach to belonging in medical education

A relational approach to medical education prioritises mentorship, peer support, and professional integration (see Table 1). These foster a sense of belonging and identity formation among medical students and trainees. Interventions such as longitudinal supervision, structured mentorship schemes, peer support networks, and transition programmes have been shown to enhance trainee confidence and well-being. The Vocational Training Scheme (VTS) provides structured peer learning, while initiatives like buddy schemes for GP trainees and the RCGP First5 programme offer informal mentorship and ongoing professional support. Similarly, Longitudinal Integrated Clerkships (LICs) in undergraduate education strengthen early professional identity by maintaining sustained exposure to general practice.

In contrast, transactional approaches focus on procedural training, formalised evaluation and standardised competency assessments such as Workplace-Based Assessments (WPBA), the Applied Knowledge Test (AKT), and Multi-Source Feedback (MSF). While essential for evaluating skills, these assessments do not actively foster professional

belonging and often fail to address the social and emotional dimensions of professional identity formation. GP trainees, in secondary care placements, may lack integration into hospital teams, with a diminished sense of connection to the broader medical workforce.

Promoting a sense of belonging

Fostering belonging requires institution-wide approaches where everyone in the learning community – learners, educators, practice staff and organisational leadership – has a key role.

Opportunities such as the induction period can be developed to promote belonging by establishing a strong foundation of quality relationships with peers, educators and practice staff. Example activities include preparing across the practice team to create a welcoming environment and sharing key information about the objectives of the placement. Spending time individually with learners at the start of the placement to explore their expectations and discuss potential challenges can help develop a mutual understanding. This can promote student agency and support tailoring high-impact learning opportunities.

Effective educator practices include promoting opportunities for learners to build connections. This could include designing small group activities in an active learning environment to facilitate formal interactions as well as having regular breaks in a social space for informal discussions. Additionally, using learner names as much as possible can have a powerful effect.

Promoting inclusivity through modelling inclusive behaviour and attitudes, and having an explicit social contract of respect helps to facilitate an environment where all learners feel valued.

Table 1. Interventions across undergraduate community clinical placement and postgraduate GP training.

Relational	Transactional
Vocational Training Scheme (VTS) – Peer support groups	Workplace-Based Assessments (WPBA) – Competency evaluations
Structured mentorship programmes – Formal mentoring by experienced GPs	Applied Knowledge Test (AKT) – Medical knowledge assessment
Buddy schemes for GP trainees – Near-peer support	Multi-Source Feedback (MSF) – Performance review based on colleague feedback
RCGP First5 initiative – Peer support and mentorship for newly qualified GPs	ePortfolio and competency sign-offs – Administrative tracking of training progress
Longitudinal Integrated Clerkships (LICs) – Continuous GP exposure during undergraduate education	Limited integration in hospital-based and GP placements
Practice-based learning and team integration – Active immersion in clinical practice	ARCP panel assessments – Formal review of training progress
Student selected modules and electives – Providing learner autonomy and accommodating flexible needs/interests	Objective structured clinical exam (OSCE) – skills assessment
Institutional focus on providing an equitable experience for students from diverse backgrounds	Institutional focus on the numbers of students from widening participation and diverse backgrounds
University academic societies – Building a community of practice	

Efforts focused on these areas can help learners to feel confident to contribute, increase their sense of belonging, and achieve their full potential.

Conclusion

Through a holistic approach to medical education, belonging can be fostered across different levels and social spaces that each build towards the sense of connection, inclusion and support. Having meaningful patient encounters and collaborating with peers and the practice staff in a psychologically safe learning environment where their contributions are valued helps promote a sense of purpose.

We propose that a culture shift towards more relational medical education can foster a greater sense of belonging. This provides a new perspective to address the current medical education and workforce challenges.

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